

D571

Psychopathology

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 34-year-old accountant who lives in a downtown loft memorizes train schedules for fun and can recite hundreds of departure times. He is happy, works well, has close friends, and harms no one. His sister thinks this hobby is “weird.” Is this behavior abnormal in the clinical sense?

- A. Yes, because a family member finds it strange.
- B. Yes, since any unusual interest is always a symptom.
- C. No; although unusual, it causes no distress, dysfunction, or danger.
- D. Yes, because the behavior is statistically rare.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

A behavior is common in a population but causes severe distress and prevents people from working. What does this show about defining abnormality?

- A. It cannot be a disorder, because it is common.
- B. It is a disorder only if it is also dangerous.
- C. Only statistical rarity defines abnormality.
- D. A common problem can still be clinically significant because of distress and dysfunction.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

Identical twins who both love hiking share a family history of depression. One twin loses her job and her partner in the same year and develops major depression; the other, whose life stays stable, does not. They were raised in a mountain town. Which model best explains the difference?

- A. Purely environmental: stress alone caused it.
- B. Diathesis-stress: a shared vulnerability became a disorder only with major stress.
- C. The mountain town protected the second twin.
- D. Purely genetic: genes alone determine depression.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

Two people face the same serious stressor. One has a strong genetic vulnerability to anxiety and the other does not. What does the diathesis–stress model predict?

- A.** The person with the vulnerability is more likely to develop an anxiety disorder.
- B.** The vulnerable person will certainly develop it.
- C.** Both will develop the disorder equally, since the stress is the same.
- D.** The person without the vulnerability is more likely to develop it.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

At a large clinic with a new electronic records system, two clinicians independently interview the same 100 patients and agree on the diagnosis 95% of the time. The clinic opened a second location last year. What does this finding establish?

- A.** High inter-rater reliability.
- B.** That the diagnoses explain what caused each disorder.
- C.** That the diagnoses are valid.
- D.** Low reliability, because 5% disagreed.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

Which finding would best support the validity of a diagnostic category?

- A.** The same clinician gives the same diagnosis twice.
- B.** The diagnosis is listed in the manual.
- C.** People given the diagnosis show a predictable course and respond to the same treatments.
- D.** Two clinicians agree on the diagnosis 90% of the time.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A patient who works as a baker is diagnosed with generalized anxiety disorder using DSM-5-TR criteria after a 45-minute interview. His brother is a firefighter. He asks, “So what caused my anxiety?” What does the diagnosis itself tell him?

- A.** That the 45-minute interview proved the exact cause.
- B.** That his symptoms meet descriptive criteria; the diagnosis does not identify the cause.
- C.** That a chemical imbalance caused it.
- D.** That childhood trauma caused it.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A college student who is majoring in art has contamination fears and washes her hands for an hour after touching doorknobs. Her roommate reassures her many times a day. She lives on the third floor of her dorm. Which treatment approach has the strongest evidence?

- A. More frequent reassurance from her roommate.
- B. Exposure and response prevention: touching doorknobs while refraining from washing.
- C. Moving to a ground-floor room.
- D. Avoiding doorknobs entirely.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A recent immigrant who runs a small tailoring shop tells a clinician that after her mother's death she felt her mother's presence and heard her voice briefly. In her community this is a common and accepted part of grieving. She is sleeping and working normally. Her shop is on a busy corner. What is the most appropriate clinical interpretation?

- A. Hearing voices always indicates psychosis.
- B. Refer her for treatment because running a business is stressful.
- C. Explore the experience in its cultural context before assuming a disorder; it may be a normal grief response.
- D. Diagnose a psychotic disorder because she hears a voice.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A popular crime drama that won several awards shows nearly every character with schizophrenia as violent. A viewer who works in marketing concludes that people with schizophrenia are dangerous. What does research say?

- A. The portrayal is accurate; most people with schizophrenia are violent.
- B. People with mental illness are never violent.
- C. Award-winning shows are carefully researched, so the portrayal is reliable.
- D. People with serious mental illness are much more likely to be victims of violence than perpetrators.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11. [TRANSFER — reversed stem / different variable]

What is a likely effect of media that repeatedly link mental illness with danger?

- A.** People become more likely to seek help, since awareness always helps.
- B.** Stigma has no measurable effect on behavior.
- C.** Diagnosis rates become more reliable.
- D.** People with symptoms become less likely to seek help because they fear being labeled.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

12.

A historian at a university with a famous library studies a period when large state psychiatric hospitals closed and patients were released to the community. New medications were available, but many towns lacked outpatient clinics and supported housing. The historian writes in a sunlit office. Which outcome is best supported?

- A.** Many people went without care, and some ended up homeless or incarcerated.
- B.** Hospital populations increased during this period.
- C.** Medications alone met all patients' needs.
- D.** Care improved for everyone, since hospitals were closed.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer C Concept 1

A 34-year-old accountant ~~who lives in a downtown loft~~ memorizes train schedules for fun and can recite hundreds of departure times. He is happy, works well, has close friends, and harms no one. ~~His sister thinks this hobby is “weird.”~~ Is this behavior abnormal in the clinical sense?

Target word: does unusual equal disordered **Decides:** No distress, dysfunction, or danger.

A	Camouflage	Another person’s opinion does not establish a disorder.
B	Absolute language	Many unusual interests are healthy.
C	Correct	Rarity alone does not meet the criteria for a disorder.
D	Dropped element	Uses one criterion and drops distress and dysfunction.

2. Answer D Concept 1 • transfer

A behavior is common in a population but causes severe distress and prevents people from working. What does this show about defining abnormality?

Target word: a common behavior that is still clinically significant — reverse case **Decides:** Distress and dysfunction matter more than frequency.

A	Inverted operation	Applies the rarity rule backward.
B	Absolute language	Danger is not required.
C	Confusion set	Rarity is one criterion among several and is neither necessary nor sufficient.
D	Correct	Frequency does not rule out a disorder; impairment does the work.

3. Answer B Concept 2

Identical twins ~~who both love hiking~~ share a family history of depression. One twin loses her job and her partner in the same year and develops major depression; the other, whose life stays stable, does not. ~~They were raised in a mountain town.~~ Which model best explains the difference?

Target word: causal model **Decides:** Same vulnerability, different stress.

A	Dropped element	Drops the family-history vulnerability.
B	Correct	Same diathesis; only the twin with heavy stress developed depression.
C	Camouflage	Both grew up there; it cannot explain the difference.
D	Absolute language	If genes alone decided, both twins would be affected.

4. Answer A Concept 2 • transfer

Two people face the same serious stressor. One has a strong genetic vulnerability to anxiety and the other does not. What does the diathesis-stress model predict?

Target word: predict outcome from stress held constant **Decides:** Vulnerability lowers the threshold.

A	Correct	A higher diathesis means less stress is needed to trigger the disorder.
B	Absolute language	The model predicts higher risk, not certainty.
C	Dropped element	Drops the difference in vulnerability.
D	Inverted operation	Reverses the effect of the diathesis.

5. Answer A Concept 3

At a large clinic ~~with a new electronic records system~~, two clinicians independently interview the same 100 patients and agree on the diagnosis 95% of the time. ~~The clinic opened a second location last year.~~ What does this finding establish?

Target word: what clinician agreement shows **Decides:** Agreement between raters.

A	Correct	Agreement between independent clinicians measures reliability.
B	Adjacent category	Agreement says nothing about cause.
C	Confusion set	Clinicians can agree consistently on a category that is not meaningful.
D	Inverted operation	95% agreement is high reliability.

6. Answer C Concept 3 • transfer

Which finding would best support the validity of a diagnostic category?

Target word: evidence for validity **Decides:** The category predicts meaningful outcomes.

A	Inverted operation	Consistency within a rater is still reliability, not validity.
B	Adjacent category	Inclusion is not evidence of validity.
C	Correct	Predictive and clinical usefulness supports validity.
D	Confusion set	That shows reliability.

7. Answer B Concept 4

A patient ~~who works as a baker~~ is diagnosed with generalized anxiety disorder using DSM-5-TR criteria after a 45-minute interview. ~~His brother is a firefighter.~~ He asks, "So what caused my anxiety?" What does the diagnosis itself tell him?

Target word: what a DSM diagnosis provides **Decides:** The DSM is descriptive, not causal.

A	Wrong stem number	Interview length does not change the descriptive nature of the diagnosis.
B	Correct	DSM categories describe symptom patterns without specifying causes.
C	Adjacent category	The manual does not assign a biological cause.
D	Confusion set	Also a causal claim the diagnosis does not make.

8. Answer B Concept 5

A college student ~~who is majoring in art~~ has contamination fears and washes her hands for an hour after touching doorknobs. Her roommate reassures her many times a day. ~~She lives on the third floor of her dorm.~~ Which treatment approach has the strongest evidence?

Target word: first-line treatment **Decides:** Compulsions maintained by rituals and reassurance.

A	Inverted operation	Reassurance acts like a ritual and maintains the anxiety.
B	Correct	Facing the cue without the ritual allows anxiety to decrease and breaks the cycle.
C	Camouflage	Her floor is irrelevant.
D	Adjacent category	Avoidance relieves anxiety short-term but strengthens the fear.

9. Answer C Concept 6

A recent immigrant ~~who runs a small tailoring shop~~ tells a clinician that after her mother's death she felt her mother's presence and heard her voice briefly. In her community this is a common and accepted part of grieving. She is sleeping and working normally. ~~Her shop is on a busy corner.~~ What is the most appropriate clinical interpretation?

Target word: culturally informed judgment **Decides:** Culturally normative grief experience with normal functioning.

A	Absolute language	Brief, culturally expected experiences in grief are not necessarily pathological.
B	Camouflage	Her business does not decide the interpretation.
C	Correct	Cultural context and intact functioning argue against pathology.
D	Dropped element	Drops cultural context and her normal functioning.

10. Answer D Concept 7

A popular crime drama ~~that won several awards~~ shows nearly every character with schizophrenia as violent. A viewer ~~who works in marketing~~ concludes that people with schizophrenia are dangerous. What does research say?

Target word: evidence on violence and mental illness **Decides:** Victimization is far more common than perpetration.

A	Inverted operation	Research shows the opposite pattern.
B	Absolute language	Risk is not zero; it is simply much lower than portrayed.
C	Camouflage	Awards do not validate the depiction.
D	Correct	The media link overstates risk and fuels stigma.

11. Answer D Concept 7 • transfer

What is a likely effect of media that repeatedly link mental illness with danger?

Target word: consequence of stigma **Decides:** Stigma discourages help-seeking.

A	Inverted operation	Negative portrayals raise fear of labeling.
B	Absolute language	Stigma affects help-seeking, employment, and housing.
C	Adjacent category	Media portrayals do not affect clinician agreement.
D	Correct	Stigma reduces treatment seeking.

12. Answer A Concept 8

A historian at a university ~~with a famous library~~ studies a period when large state psychiatric hospitals closed and patients were released to the community. New medications were available, but many towns lacked outpatient clinics and supported housing. ~~The historian writes in a sunlit office.~~ Which outcome is best supported?

Target word: effect of deinstitutionalization without supports **Decides:** Too few community services after hospital closures.

A	Correct	Hospital closures without enough community services left gaps in care.
B	Inverted operation	Deinstitutionalization reduced hospital populations.
C	Dropped element	Drops housing, follow-up, and social support.
D	Absolute language	Outcomes depended on community services, which were often lacking.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	5	
Wrong stem number	A figure copied from the prompt that is not the answer.	1	
Adjacent category	The right idea in the wrong bucket.	5	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	8	
Absolute language	Always, never, guarantee — where judgment applies.	8	
Confusion set	Every option is real; the test is exact role-matching.	4	
Camouflage	Answers a stem fact that was never the question.	5	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Defining abnormality	Abnormality is judged by several criteria together—deviance from norms, distress, dysfunction in daily life, and sometimes danger—not by statistical rarity alone.	Calling any rare or unusual behavior a disorder.	A behavior that is unusual but causes no distress or impairment.
2	Diathesis-stress model	A disorder develops when a predisposition (diathesis: genetic, biological, or psychological vulnerability) combines with enough environmental stress; neither alone is usually sufficient.	Assuming a vulnerability guarantees the disorder, or that stress alone causes it in everyone.	A family history plus a major life stressor.
3	Reliability vs. validity of diagnosis	A diagnosis is reliable if different clinicians agree using the same criteria; it is valid if it captures a real, meaningful condition that predicts course or treatment response.	Treating high clinician agreement as proof the category is valid.	Two clinicians agree—or disagree—on a diagnosis.
4	Classification systems	The DSM-5-TR and ICD are mainly categorical and descriptive: they define disorders by observable symptom criteria rather than by cause; dimensional approaches rate symptoms along continua.	Believing a DSM diagnosis explains why the disorder occurred.	A question about what a diagnosis does or does not tell you.
5	Exposure-based treatment	Exposure therapy, including exposure and response prevention for OCD, is a first-line evidence-based treatment for phobias and OCD: the person faces feared cues without avoidance or rituals until anxiety decreases.	Thinking avoidance or reassurance reduces anxiety long-term.	Fear or compulsions maintained by escape, avoidance, or rituals.
6	Cultural concepts of distress	Culture shapes how distress is experienced, expressed, and explained; clinicians use tools such as the Cultural Formulation Interview to avoid over- or under-diagnosing.	Labeling a culturally normal expression of distress as psychosis.	Symptoms described in terms of spiritual, bodily, or community meaning.
7	Stigma and media depictions	Media often link mental illness to violence, but people with serious mental illness are far more likely to be victims than perpetrators of violence; such depictions fuel stigma and discourage help-seeking.	Accepting the violent stereotype as accurate.	A news or film portrayal of a “dangerous” person with mental illness.

#	Concept	Rule	Trap	Tell
8	Historical shifts in explanation and care	Views moved from supernatural to somatogenic (bodily cause) and psychogenic (psychological cause) explanations; moral treatment and later deinstitutionalization changed where care happened, and deinstitutionalization without enough community services contributed to homelessness and incarceration.	Assuming deinstitutionalization alone improved care for everyone.	Reforms in where and how people with mental illness were treated.

Course scope (public description): Psychopathology examines historical and contemporary views and issues of abnormal behavior. Students are provided with an overview of the ways mental health practitioners explain, diagnose, and treat behavioral disorders. Sociocultural impacts on abnormal behavior, including the manifestation, categorization, and treatment of mental illness, are explored. Biases and popular depictions of mental health and mental illness are analyzed. There are no prerequisites for this course.